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Susan Stonex-Carroll

Assessment of ANS function during Valsalva Maneuver combined with Deep Breathing

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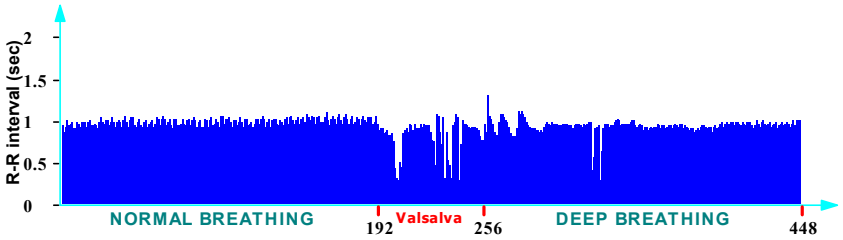
Electrocardiographic Rhythm Strip



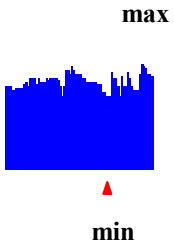
HR	R(HF)	R(LF1)	R(LF2)
60	16	29	29

Valsalva Maneuver

HR	R(HF)	R(LF1)	R(LF2)
62	25	28	27

Extrasystoles analysis

Extrasystoles not recognized



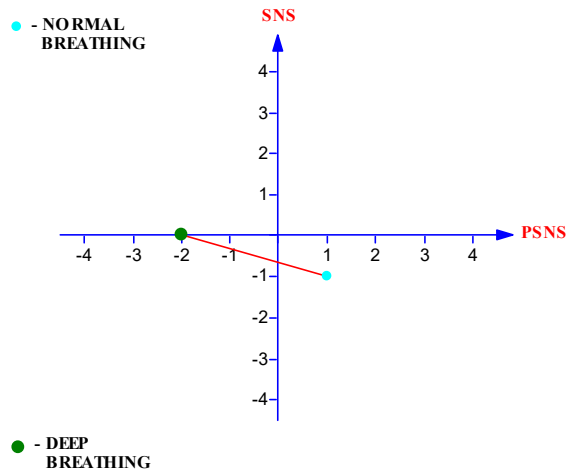
- Athletic level (>1.7)
- Average level (1.3 - 1.7)
- Low level (<1.3)

Valsalva Ratio 1.41

E/I Ratio 1.06

ANS ASSESSMENT (NORMAL BREATHING)	ANS ASSESSMENT (DEEP BREATHING)
INFLUENCE OF PARASYMPATHETIC SYSTEM PREVAILS MODERATELY WHILE SYMPATHETIC IS DECREASED SLIGHTLY	PARASYMPATHETIC SYSTEM IS DECREASED MODERATELY WHILE SYMPATHETIC ACTIVITY ON AVERAGE LEVEL

GRAPHICAL PRESENTATION OF THE ANS CONDITION



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For Data Interpretation only, NOT A DIAGNOSIS; must be interpreted by a Physician

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Interpretation of ANS assessment based on Heart Rate Variability

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Orthotest	Valsalva Maneuver combined with Deep Breathing
There is a moderate PSNS Hyper-Regulation in response to the orthostatic intervention.	Deep breathing vs. Normal breathing shows negative changes by the Parasympathetic activity. Deep breathing vs. Normal breathing slightly elevates the Sympathetic activity. Valsalva ratio is on an average level.

Summary

Valsalva maneuver combined with Deep breathing shows a slight reserve of the Sympathoadrenergic function.
Baroreceptors sensitivity is on an average level.
Autonomic function is close to the normal physiologic range.
There is a normally trained person
Moderate or severe PSNS Hyper-Regulation during orthostatic test is related for up to 80% of cases with gastrointestinal(GI) system problems and up to 20% with syncope, dizziness and vestibular deregulation.
NOTE: High correlation of the PSNS imbalance with GI system problems is based on a highly involving Vagus nerve activity in GI system innervations.

* Final decision can be made ONLY by combining ANS assessment conclusions with clinical data!

Doctor's name: _____ Doctor's signature: _____